

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Postnatal Contraception for the treatment of Postnatal Contraception

Patient Group Direction, version 004, issued 11 September 2026. Supersedes Version 003, 10 September 2026.

Summary of the governing guidance for postnatal contraception: CoSRH (formerly FSRH) clinical guideline Contraception After Pregnancy (2017, amended 2020), UKMEC 2025, and the SmPCs of the products supplied. NICE CKS 'Contraception - assessment' summarises the same sources.

Overview

- Fertility can return as early as 21 days post-delivery.
- Contraception advice should be provided during postnatal care.
- Progesterone-only methods are safe during breastfeeding.

Assessment

- Delivery date
- Breastfeeding status
- Medical history
- VTE risk
- Previous contraceptive use
- Postnatal complications

Management

- Progesterone-only pill (POP) can start any time postpartum (day 1 or later).
- Depo-Provera from 6 weeks postpartum if breastfeeding; from 21 days if not breastfeeding and no additional VTE risk factor.
- UKMEC criteria apply.

Red flags - seek urgent medical advice

- DVT/PE symptoms
- Breast cancer (current)
- Severe liver disease
- Unexplained vaginal bleeding

Patient Group Direction

for the supply/administration of
Desogestrel 75mcg tablets (Cerazette/Cerelle)
for the treatment of
Postnatal Contraception

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Postnatal contraception for women aged 16+
Inclusion criteria	Postnatal women aged 16 years and over Any time postpartum. Before day 21 no additional contraception is needed. From day 21, pregnancy must be reasonably excluded (no unprotected intercourse since day 21, or a negative test 21 days after the last episode) and 2 days of extra precautions advised (FSRH; the SmPC gives 7 days). Informed consent obtained UKMEC 1 or 2 criteria met
Exclusion criteria	Known or suspected pregnancy Active thromboembolic disorder Sex hormone-dependent malignancy (current or suspected breast

	cancer) Severe hepatic impairment or liver tumours Undiagnosed vaginal bleeding Hypersensitivity to desogestrel or any excipients
Cautions	History of breast cancer (more than 5 years ago) Functional ovarian cysts Diabetes Hypertension Migraine Depression
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Desogestrel 75 micrograms tablets
Legal category	POM
Storage	Store below 30°C
Route/method of administration	Oral, one tablet daily
Dose and frequency	One tablet daily at the same time each day, continuously (no pill-free break). 12-hour window for missed pills. Start any time postpartum. Started up to and including day 21: no additional contraception needed. Started after day 21: exclude pregnancy first and use a barrier method for 2 days (FSRH; the SmPC states 7 days).
Quantity to be administered and/or supplied	Up to 3 months' supply (3 x 28 tablets = 84 tablets)
Maximum or minimum treatment period	Continuous until change of contraception or pregnancy
Adverse effects	Common: Irregular bleeding or spotting, headache, mood changes, breast tenderness, nausea, acne, decreased libido, weight gain Uncommon: Alopecia, ovarian cysts, fatigue

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk
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	and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication. Discuss breastfeeding safety and the delayed return of fertility with Depo-Provera.
Follow-up advice to be given to patient or carer	Discuss and explain that irregular bleeding is common with both methods, particularly in the first few months. With Depo-Provera, explain that fertility may take 5-6 months to return after the last injection. Both methods are safe during breastfeeding. Take the tablet at the same time each day (Desogestrel) to maintain effectiveness. Return for repeat injection every 12 weeks (Depo-Provera). Seek medical advice if symptoms worsen or any adverse effects occur. Seek immediate medical attention if symptoms of DVT/PE develop (calf pain, swelling, breathlessness). Report any unexpected vaginal bleeding. Discuss contraceptive options if considering longer-term contraception.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017
<https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017
<https://www.nice.org.uk/guidance/mpg2/resources>
- College of Sexual and Reproductive Healthcare (formerly FSRH), Contraception After Pregnancy, January 2017 (amended October 2020); UKMEC 2025; Summaries of Product Characteristics for the products supplied, current versions
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
11/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Medroxyprogesterone acetate 150mg/ml injection (Depo-Provera)

for the treatment of

Postnatal Contraception

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Postnatal injectable contraception for women aged 18+
Inclusion criteria	Postnatal women aged 18 years and over From 6 weeks postpartum if breastfeeding. From 21 days postpartum if NOT breastfeeding and there is no additional VTE risk factor (previous VTE, thrombophilia, immobility, BMI 30 or over, caesarean delivery, postpartum haemorrhage, pre-eclampsia, smoking). Otherwise refer. Informed consent obtained UKMEC 1 or 2 criteria met
Exclusion criteria	Known or suspected pregnancy Current breast cancer

	Severe liver disease Undiagnosed vaginal bleeding History of hypersensitivity to medroxyprogesterone acetate or any excipients Severe cardiovascular disease Meningioma, current or previous
Cautions	Bone mineral density loss with prolonged use (review at 2 years) Weight gain Delayed return of fertility (median 5-6 months after last injection) Breast cancer history Migraine Diabetes Depression Adolescents (bone mineral density concern)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Medroxyprogesterone acetate 150 milligrams per millilitre injection
Legal category	POM
Storage	Store below 25°C. Do not freeze.
Route/method of administration	Deep intramuscular injection into gluteal or deltoid muscle
Dose and frequency	150 milligrams by deep intramuscular injection Repeat every 12 weeks (± 5 days) First injection from 6 weeks postpartum if breastfeeding; from 21 days if not breastfeeding and no additional VTE risk factor (see inclusion criteria). Otherwise refer.
Quantity to be administered and/or supplied	Single injection per visit
Maximum or minimum treatment period	Every 12 weeks (± 5 days) until change of contraception or pregnancy
Adverse effects	Very common: Menstrual irregularity, weight gain Common: Headache, dizziness, mood changes, acne, abdominal

	discomfort, decreased libido Long-term: Reduced bone mineral density (usually reversible)
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication. Discuss breastfeeding safety and the delayed return of fertility with Depo-Provera.
Follow-up advice to be given to patient or carer	Discuss and explain that irregular bleeding is common with both methods, particularly in the first few months. With Depo-Provera, explain that fertility may take 5-6 months to return after the last injection. Both methods are safe during breastfeeding. Take the tablet at the same time each day (Desogestrel) to maintain effectiveness. Return for repeat injection every 12 weeks (Depo-Provera). Seek medical advice if symptoms worsen or any adverse effects occur. Seek immediate medical attention if symptoms of DVT/PE develop (calf pain, swelling, breathlessness). Report any unexpected vaginal bleeding.

	Discuss contraceptive options if considering longer-term contraception.
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Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
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Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The guidance summary and references name CoSRH Contraception After Pregnancy and UKMEC 2025 as the governing sources. There is no NICE CKS topic on postnatal contraception.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

Depo-Provera arm, exclusion added: meningioma, current or previous. The Depo-Provera SmPC (September 2025) contraindicates medroxyprogesterone in patients with meningioma or a history of meningioma; the previous version did not mention it.

Version: v003

Issued: 10 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Desogestrel: initiation allowed at any time postpartum, as the document's own guidance summary and UKMEC 2025 (category 1 at every postpartum interval) say. The previous inclusion and dose row required day 21. Starts after day 21 need pregnancy excluded and 2 days of extra precautions (FSRH); the SmPC's 7 days is noted.

Depo-Provera: one early-start rule in place of three. From 6 weeks if breastfeeding; from 21 days if not breastfeeding and no additional VTE risk factor; otherwise refer. 'Per clinical judgement' is gone: it is not an auditable PGD criterion.

Signed on behalf of Get Real Health

Version v004, 11 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		11 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		11 September 2026

Both authorising signatories reviewed this version together on 11 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.